

Induction of labor

For 22.3 percent of women in the US, labor is started by artificial means, or induced, to reduce the risks to mother and baby.

An induction may be offered if it's felt that continuing the pregnancy poses a risk to your health or to the health of your baby. The most common reason for an induction is the continuation of a pregnancy beyond 41 or 42 weeks, in which case the placenta may begin to fail. Induction may be offered earlier if you are having twins, or have a medical condition such as diabetes. Before setting a date for induction, your doctor may offer to [strip your membranes](#) to help you go into spontaneous labor.

Induction is not the same as an augmentation of labor, which is when drugs are used to increase the efficiency of your contractions when you've already gone into labor spontaneously (see [How is labor sped up with drugs?](#)).

Assessing the cervix

Before an induction, you'll have an internal examination to assess the cervix. Induction is easier if your cervix is short and soft, described as "favorable" or "ripe," rather than long and firm. The findings may be logged in a table called the Bishop's Score, which also assesses how far the cervix is [dilated](#), the position of the cervix, and the station of the fetal head in the pelvis (see [Changes to the cervix](#)). A total score over six indicates good conditions for an induction of labor.

Softening the cervix

If the cervix isn't ripe, it can be softened with prostaglandins. These are naturally occurring chemicals that help stimulate contractions. Artificial prostaglandins can be given in the form of vaginal tablets of gel, which are placed at the top of the vagina near the cervix, or a vaginal suppository or an oral pill. This is usually effective, but sometimes prostaglandins fail to soften the cervix and may be tried again after a few days. On the other hand, some women experience dramatic effects after a small dose.

Breaking the water

Artificial rupture of the membranes (ARM) is one of the most important steps in the induction process, often referred to as "breaking the water." It's done once the cervix is soft and slightly dilated, and the head has started to enter the pelvis. A thin plastic probe is passed through your cervix and used to make a small hole in the amniotic membranes, which allows some of the fluid around your baby to leak out. This softens the cervix even more and can provoke contractions in the wall of the uterus. If contractions don't become established after ARM, then you'll require treatment with oxytocin.

Oxytocin

Oxytocin is a natural hormone that stimulates the uterus, increasing the frequency and strength of contractions. A synthetic form, Syntocinon or Pitocin, is used with the same effect. It's diluted then put in an IV in your arm or injected into a large muscle. This is safe and effective when used correctly; however, it must be used with care since excessive contractions can reduce your baby's oxygen supply in labor. Your contractions and your baby's heartbeat will be continuously monitored (see [External electronic fetal monitoring](#)) throughout an induction of labor.